

DETAILED PRACTICAL GUIDE

Clinical management of rape and intimate partner violence survivors

WHO, UNFPA and UNHCR · English · 2020

Important: This is a detailed unofficial reading guide, not an official translation. Consult the original publication for complete requirements and context.

Based on the original publication

<https://iris.who.int/bitstream/handle/10665/331535/9789240001411-eng.pdf>

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01 Purpose and scope

- Clinical guidance for compassionate, evidence-based care after rape and intimate-partner violence, including urgent treatment, documentation and follow-up.

02 Key guidance

- Address life-threatening conditions first while preserving dignity, privacy and the survivor's control over each examination step.
- Obtain informed consent separately for examination, evidence collection, treatment, photographs and information sharing; consent may be withdrawn.
- Offer time-sensitive pregnancy prevention, HIV post-exposure prophylaxis and sexually transmitted infection care according to current national protocols.
- Do not perform virginity testing; it has no scientific validity and can cause physical and psychological harm.
- Use trauma-informed examination: explain each step, minimise repetition, allow a support person if desired and stop when asked.
- Forensic evidence collection must follow local law, chain-of-custody rules and available laboratory capacity; medical care must never depend on evidence collection.
- Assess mental health, acute stress, self-harm risk and social needs without pathologising expected trauma responses.
- Plan follow-up for test results, vaccinations, medication adherence, pregnancy, healing, mental health and safe referral.

03 How to use this guide

- This guide turns the source publication into a practical reading aid. It is designed for survivors, trusted supporters, health and social-service staff, and programme managers. It does not replace emergency services, medical care, legal advice, or the complete source publication.
- Use only the parts that fit the person's situation. Preserve her choice, privacy and pace. If a suggested action could expose browsing, calls, documents or location to an abusive person, first choose a safer device, place or time.

04 Survivor-centred principles

- Safety comes first. Ask what would make the next hours or days safer and avoid steps that may provoke retaliation. Never require a disclosure in order to offer basic help.
- Respect autonomy and informed consent. Explain realistic options, benefits, limits and possible risks; the survivor decides what happens next unless an immediate legal safeguarding duty applies.
- Protect confidentiality. Collect the minimum information, restrict access, agree on safe contact methods and explain any limits before information is shared.
- Provide equitable access. Consider disability, language, age, migration status, poverty, rural isolation, sexual orientation and other barriers without stereotyping.

05 Practical response sequence

- Begin with immediate safety and urgent health needs. Ask privately, without the suspected perpetrator present, whether it is safe to talk and whether the person needs emergency care.
- Listen without interrogation. Validate what happened, avoid blame, and do not demand a chronological account or proof. Record only what is necessary and use the survivor's own words.
- Offer a small number of verified options: medical care, safe accommodation, psychosocial support, legal information, police or protection services where appropriate. Explain that accepting one service does not require accepting all others.
- Make a warm referral when consent is given: confirm the service is open, accessible and safe; explain what to expect; and avoid sending the survivor repeatedly between agencies.
- Agree on follow-up and a safe way to communicate. Do not leave identifying messages, send revealing notifications or store files under obvious names if devices may be monitored.

06 Safety and quality checks

- Do not confront the suspected perpetrator, mediate a violent relationship, promise an outcome you cannot control, or pressure a survivor to leave immediately.
- Do not use unverified phone numbers or referrals. Services may close, change eligibility rules or share information in unsafe ways; confirm details before publication or referral.
- Organisations should train staff, supervise difficult cases, create confidential documentation rules, maintain updated referral maps and offer staff support to reduce burnout and harmful practice.

Action checklist

- Is it safe to speak, read or keep this document right now?
- Have urgent injuries, pregnancy concerns, sexual-violence care and suicide risk been considered?
- Has the survivor chosen what information may be recorded or shared?
- Are interpretation, disability access, childcare, transport and cost barriers addressed?
- Are referrals verified and is there a safe follow-up plan?
- Has the person been told that the violence is not her fault and that she can decline any option?